

Early Warning Score and Deteriorating Patient Escalation

Policy Number	POL-CLN-041	Owner / Department	Nursing / Patient Safety
Effective Date	2023-11-06	Version	2.3
Last Revised	2026-02-18	Next Review	2028-02-18
Approved By	Patient Safety & Quality Committee	Applies To	All nursing and medical staff caring for adult inpatients on general wards and in step-down units

1. Purpose

To detect clinical deterioration early and trigger a timely, graded clinical response using a standardized aggregate track-and-trigger early-warning score. Riverside uses the Royal College of Physicians National Early Warning Score (NEWS2) for adult inpatients.

2. Scope

Applies to all adult inpatients except those on an agreed palliative/comfort-care pathway. NEWS2 is calculated at every set of vital signs and whenever clinical concern arises; it does not replace clinical judgment, and a worried clinician may escalate regardless of score.

3. The NEWS2 Score

NEWS2 aggregates seven physiological parameters: respiratory rate, oxygen saturation (with a separate scale for patients with target range 88–92%), supplemental oxygen, temperature, systolic blood pressure, pulse, and level of consciousness (ACVPU). Each parameter scores 0–3; a score of 3 in any single parameter is itself a trigger.

4. Escalation Triggers and Response

IMPORTANT — A NEWS2 aggregate score of 7 or more, or new confusion / ACVPU other than Alert, is an emergency: immediately call the Rapid Response Team (dial 66), begin continuous monitoring, and do not leave the patient unattended. A patient scoring ≥ 5 requires urgent clinical review within 30 minutes.

NEWS2 score	Risk / frequency	Response
0	Low; minimum 12-hourly.	Continue routine monitoring.
1–4	Low; minimum 4–6-hourly.	Registered nurse assesses; decide whether to increase frequency or escalate.
3 in any one parameter	Low–medium; minimum hourly.	Urgent review by the ward doctor; consider escalation.

NEWS2 score	Risk / frequency	Response
5–6	Medium; minimum hourly.	Urgent review by a clinician within 30 min; consider Rapid Response Team.
7 or more	High; continuous monitoring.	Emergency: activate Rapid Response Team (dial 66); critical-care assessment.

5. Documentation Requirements

- Record all seven parameters, the aggregate score, and the monitoring frequency on the observation chart.
- Document the time of each escalation, who was contacted, and the response/plan.
- Note any clinician decision to vary monitoring frequency from the protocol, with the rationale.

6. Severity & Reporting

Report escalation failures per POL-RM-013. A failure to recognize or act on a rising score that leads to death or permanent harm is a failure-to-rescue sentinel event.

Severity	Example	Notify / timeframe
SEV-1	Documented high score not escalated; unrecognized deterioration leading to death or permanent harm (sentinel event).	Attending, Administrator-on-Call, and Risk Management within 1 hour.
SEV-2	Escalation delayed beyond protocol with major temporary harm.	Unit manager and Risk Management within 4 hours.
SEV-3	Missed or late observation set with no harm.	Unit manager within 24 hours via event report.
Near Miss	High score caught and escalated promptly; no harm reached the patient.	Event report within 24 hours.

7. References

- Royal College of Physicians, National Early Warning Score (NEWS2), updated report of a working party.
- Related early-warning scoring literature (MEWS).
- Related: POL-CLN-040, POL-RM-013.